

The Episode, End to End

Referral to admission · plan of care · the clinician's own week · recertify or discharge

- Intake

Clinician
- Insurance & Auth

HCHB
- PCC / Scheduler

Per Diem / Float
- DCS

Branch Leadership

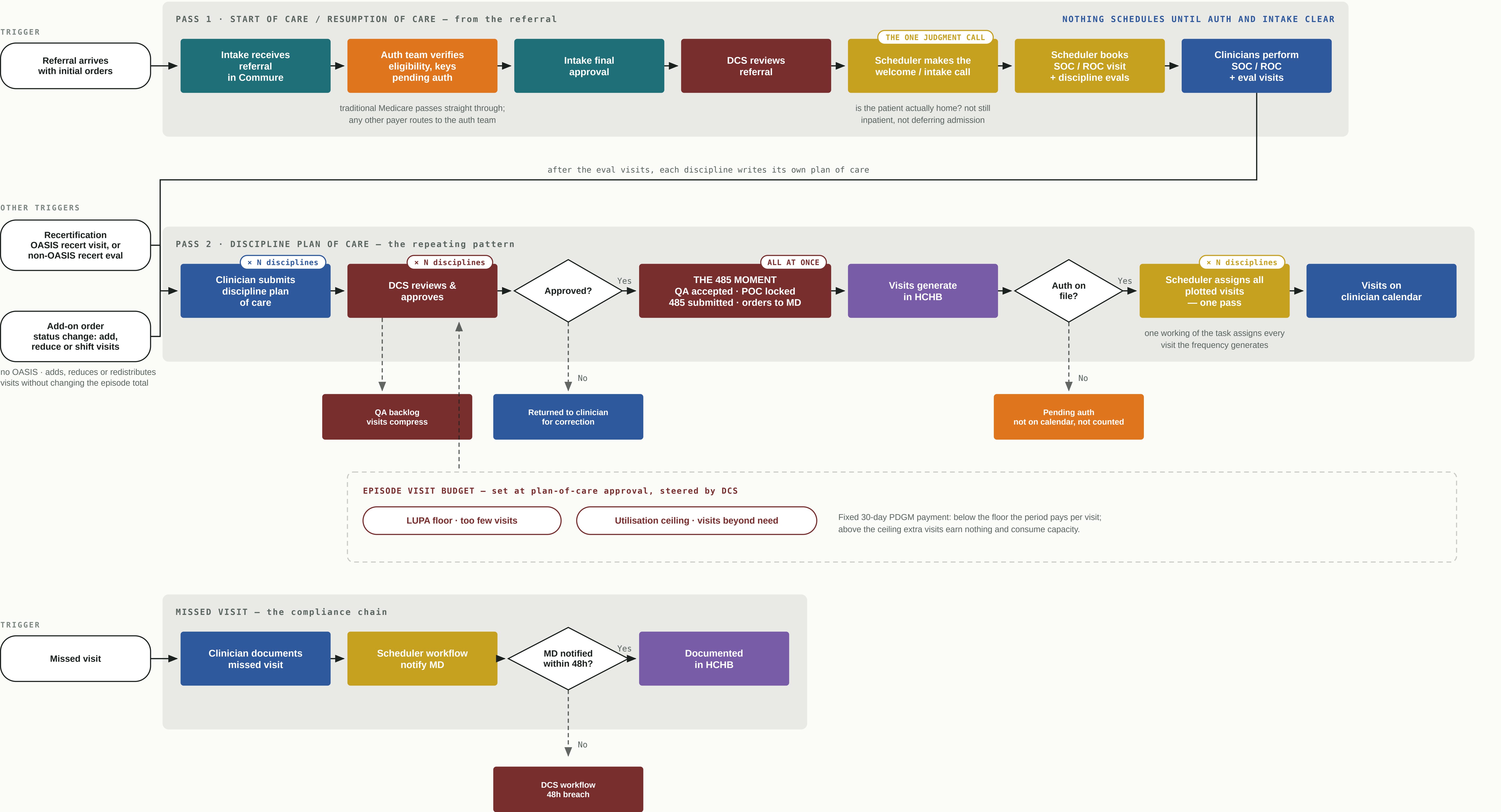


Start of Care — the Full Flow

From the referral through the plan of care to visits on the calendar — SOC · ROC · recert · add-on orders

Intake Insurance & Auth DCS PCC / Scheduler Clinician HCHB

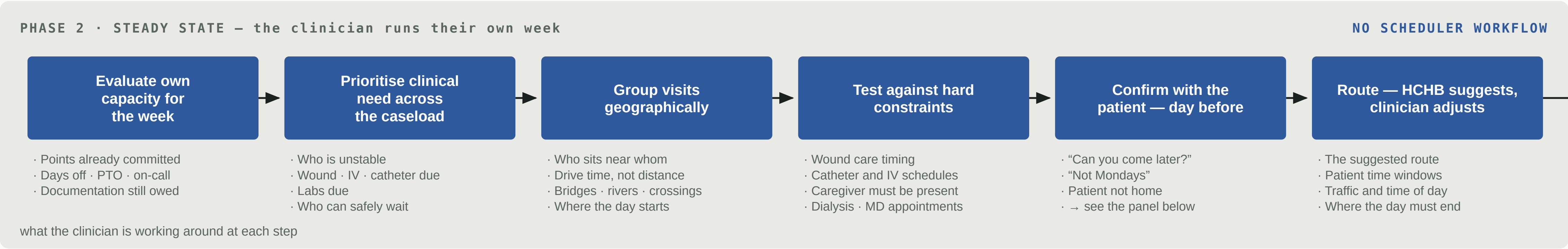
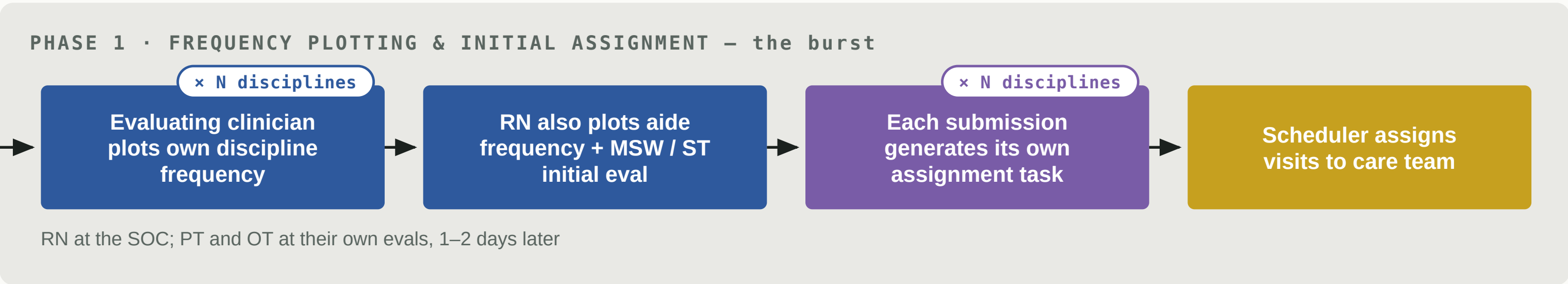
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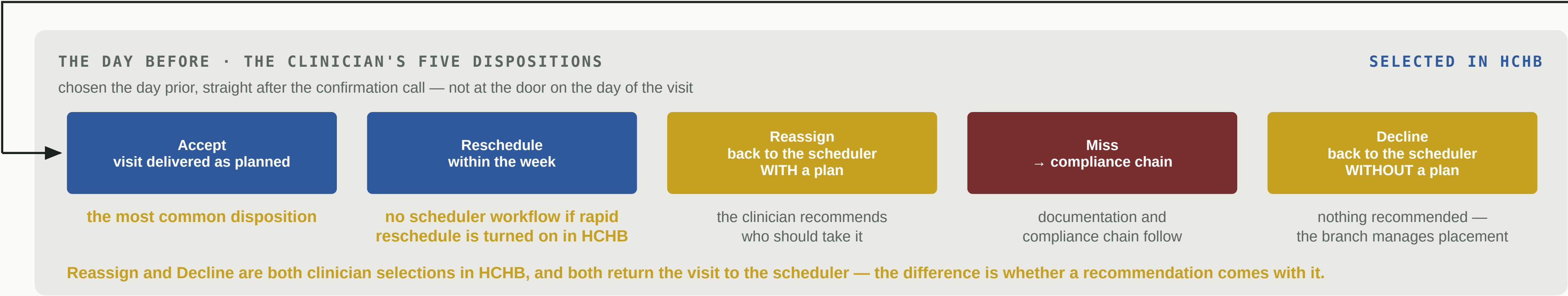
Routine Visit Scheduling

A burst of scheduler work at admission, then the clinician runs their own week

TRIGGER



The first visit at 8 or 9am is the single largest lever on an individual clinician's capacity.
Newer clinicians let the patient set the time, become over-accommodating, and push the cost onto the rest of the team.



BOUNDARIES

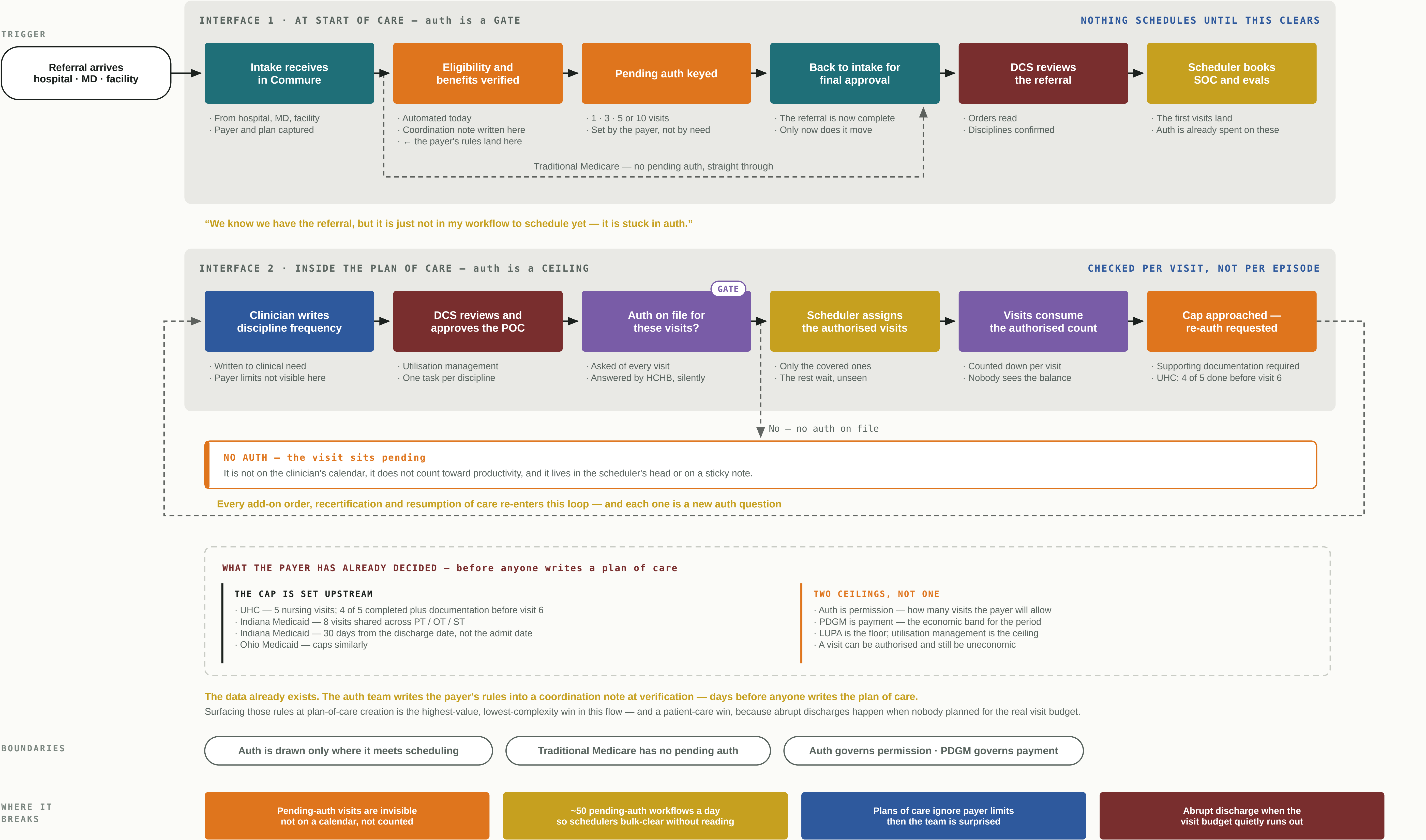


WHERE IT
BREAKS



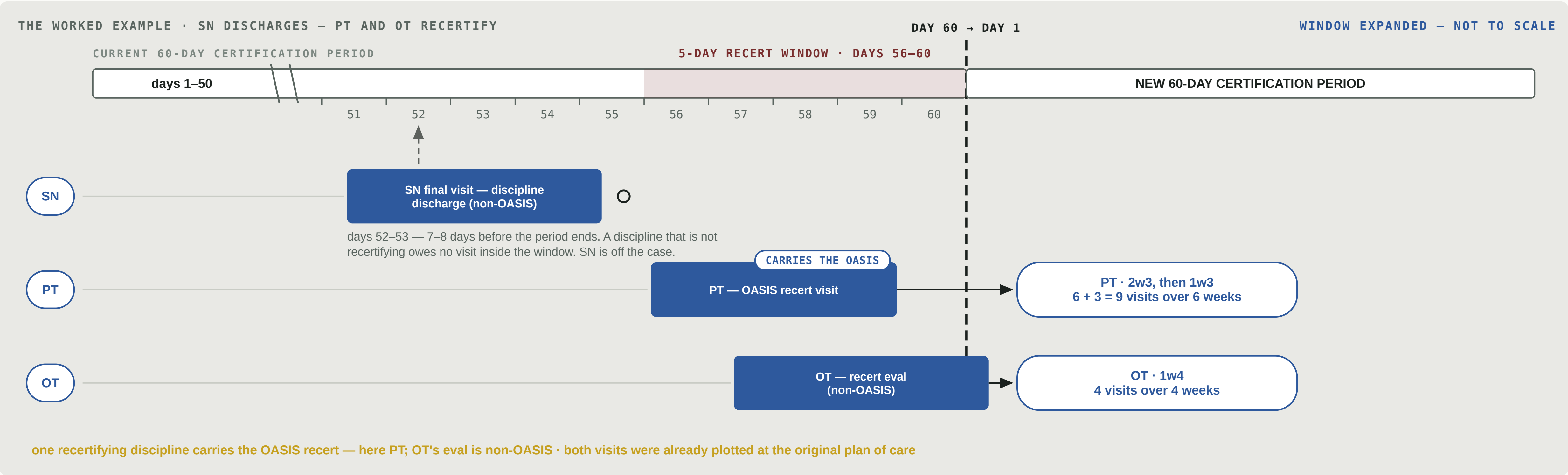
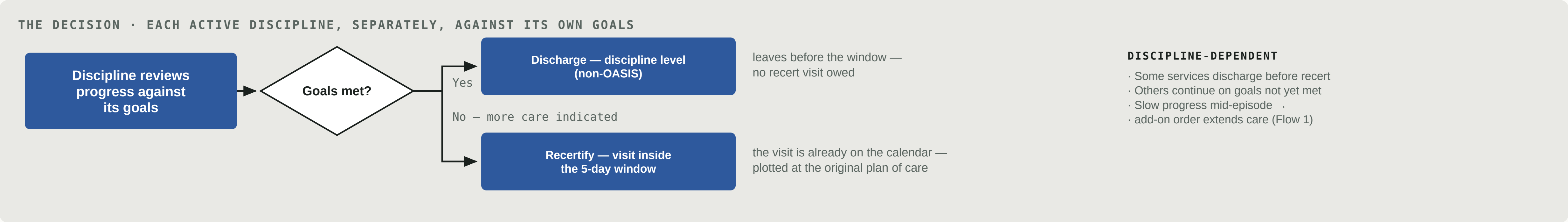
Authorization

Two interfaces only — the gate before the first visit, and the ceiling inside the episode



Recertification & Discharge

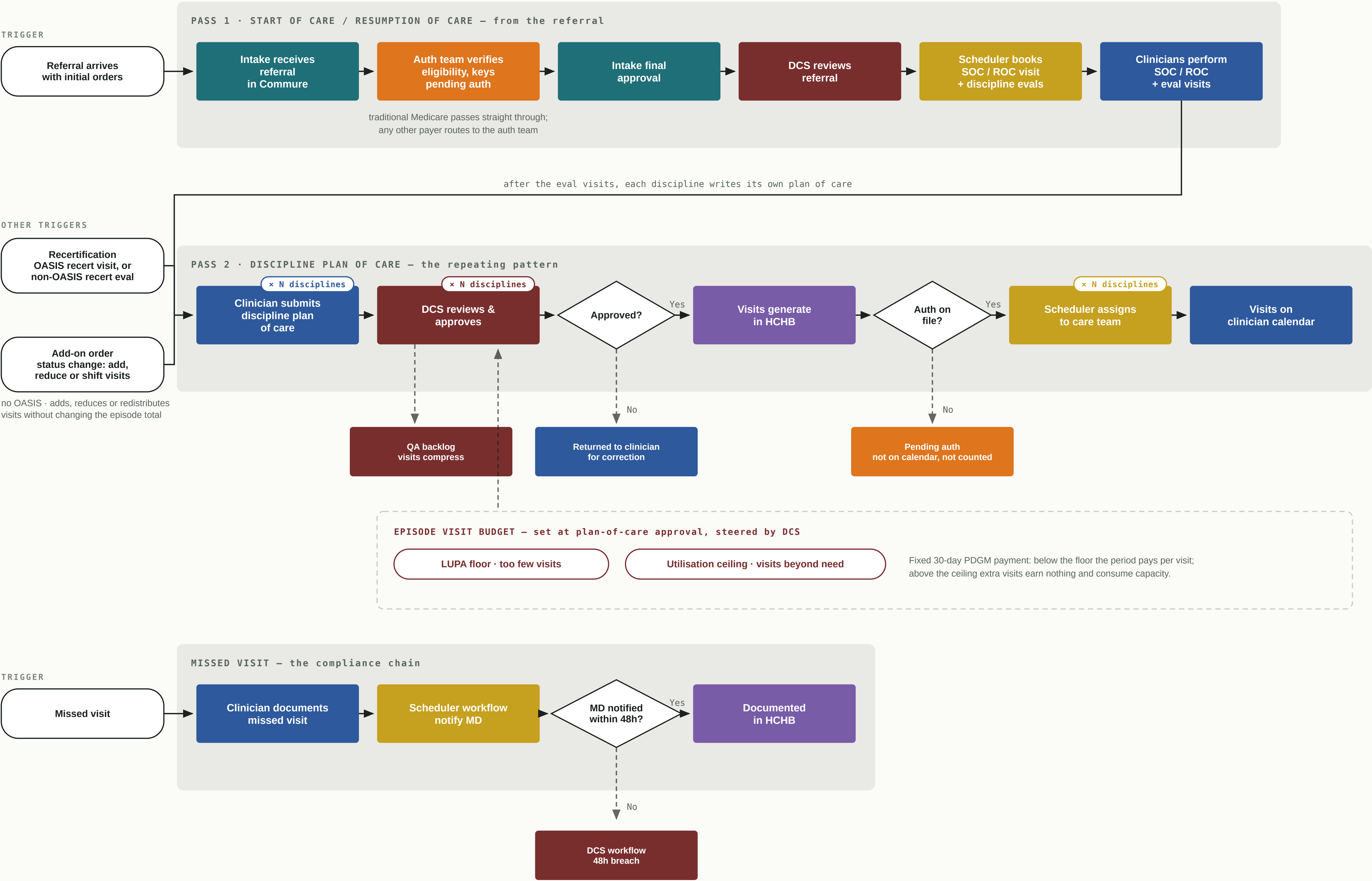
Each discipline decides for itself — and the last one out does the OASIS



Plan of Care → Assignment

The DCS and scheduler handoff — a simplified subset of the full swimlane

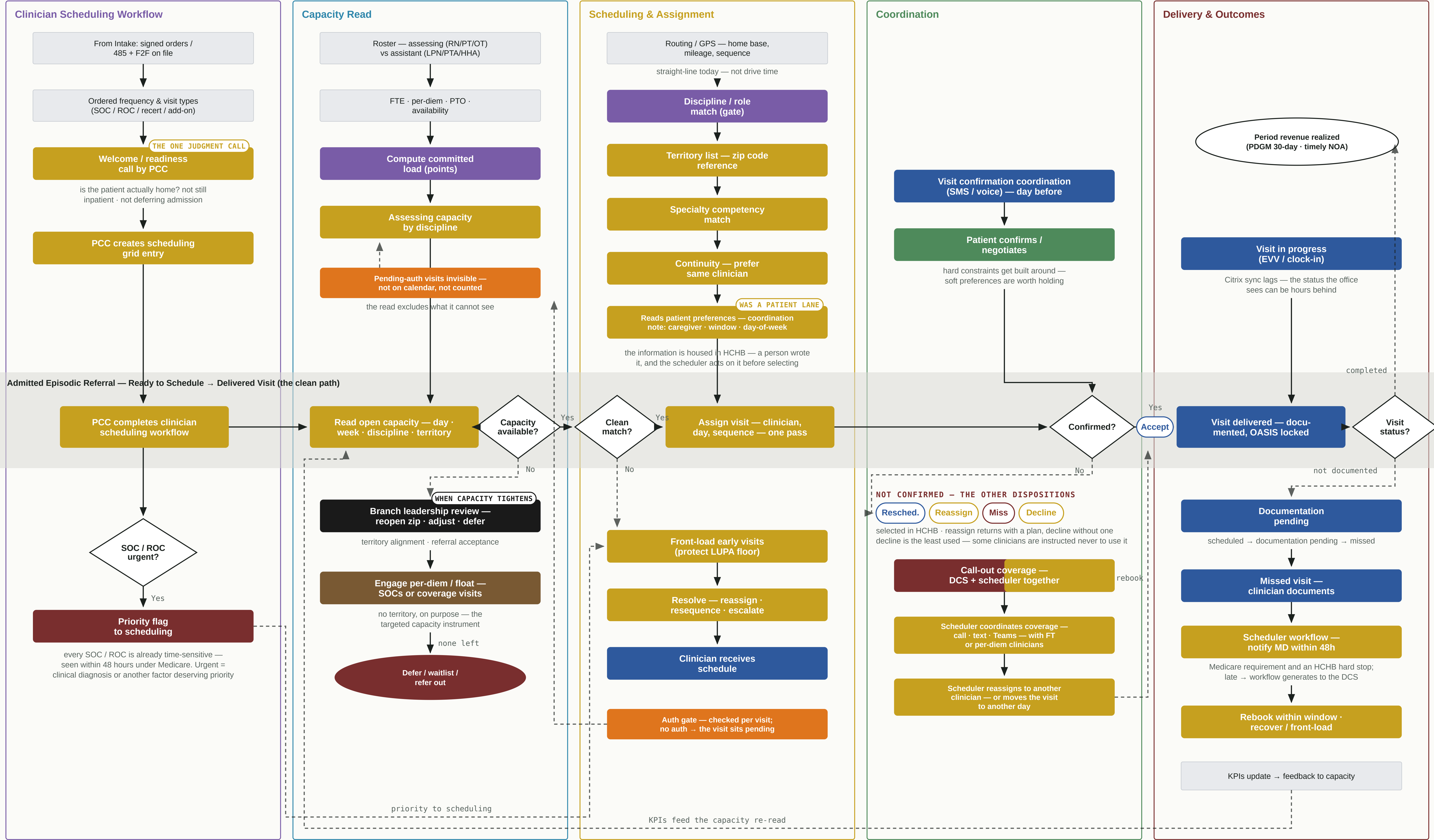
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Home Health Capacity & Scheduling — Detailed Flow

Continues the Home Health Intake Reset — from the PCC clinician scheduling workflow forward · current state, corrected 18 Aug 2026

PCC (Scheduler) HCHB ED / DCS Clinician Per Diem / Float Patient Insurance & Auth Branch Leadership



One referral, one episode

Friday afternoon to the close of the first payment period. Four points where money leaves.

- Intake

Insurance & Auth

PCC / Scheduler

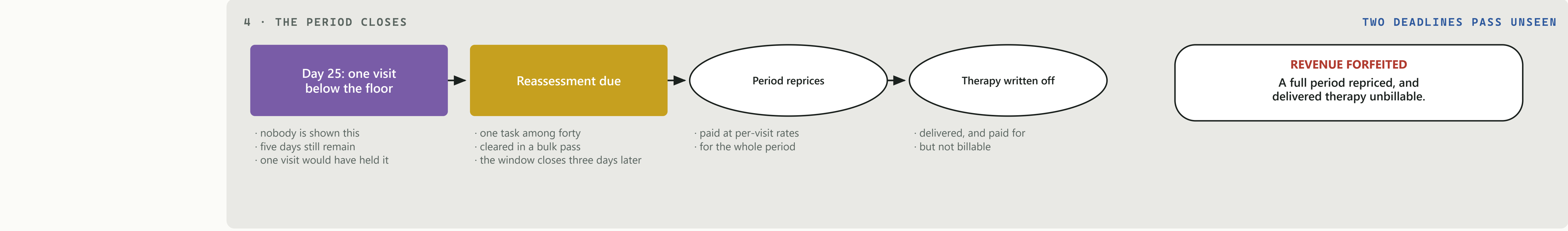
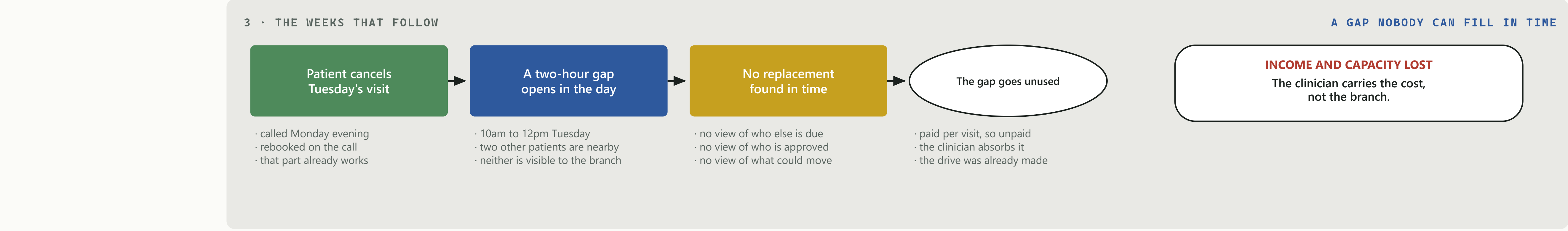
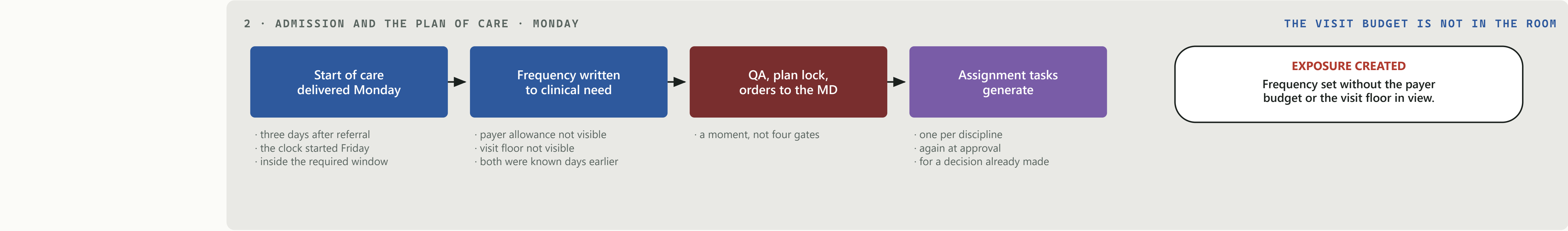
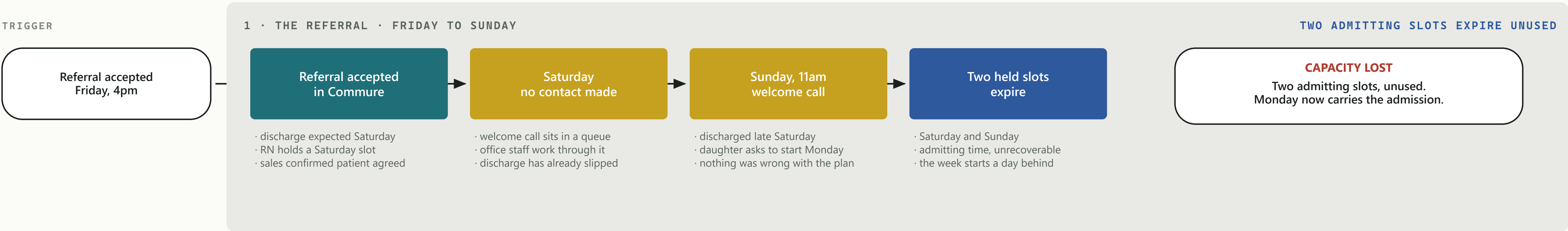
Clinician

DCS

HCHB

Patient

TRIGGER



WHAT THIS EPISODE COST, IN ORDER

1 ADMITTING TIME

- two slots expired before
- the patient was ready

2 THE PLAN OF CARE

- written blind to the payer
- budget and the visit floor

3 THE CANCELLED VISIT

- a gap the branch could not
- fill inside the day

4 THE TWO DEADLINES

- both passed unseen, both
- cost real money

LEVERS ON THIS SHEET

- L1 Admission Throughput
- L3 Same-Day Schedule Recovery
- L4 Episode Payment Protection
- L5 Reassessment Window Compliance

One referral, one episode — the same week, after

The same patient, the same decisions they make, and what changes around them.

- Intake

Insurance & Auth

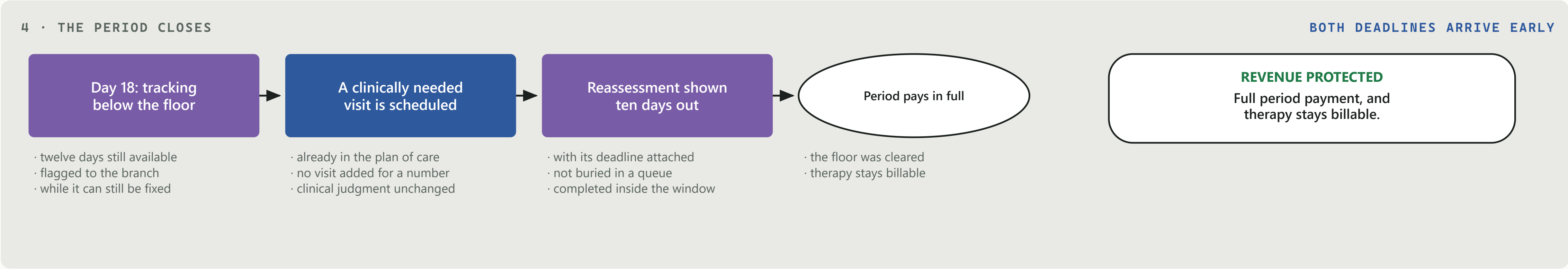
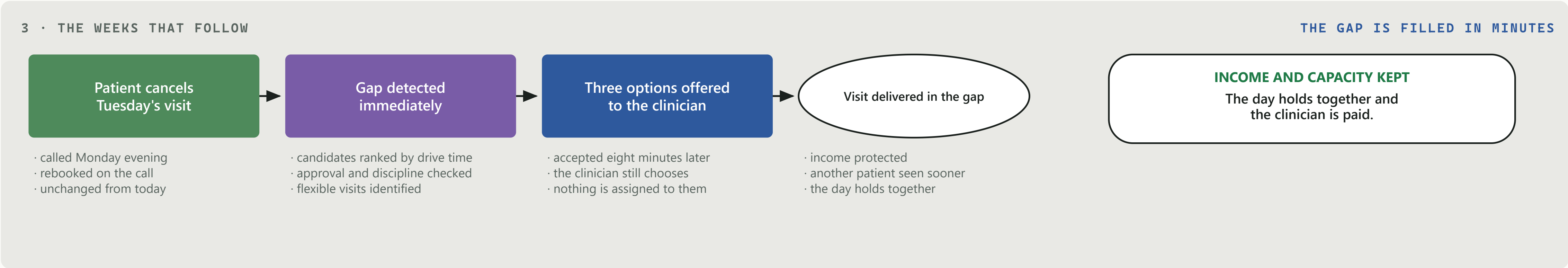
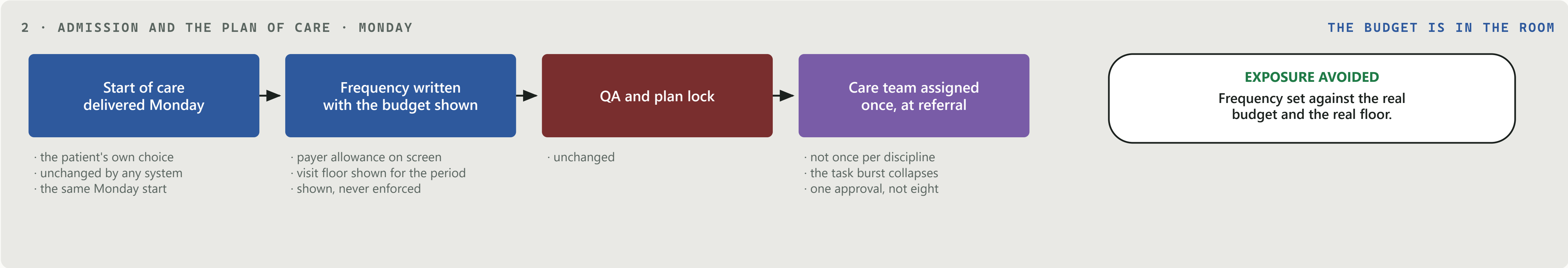
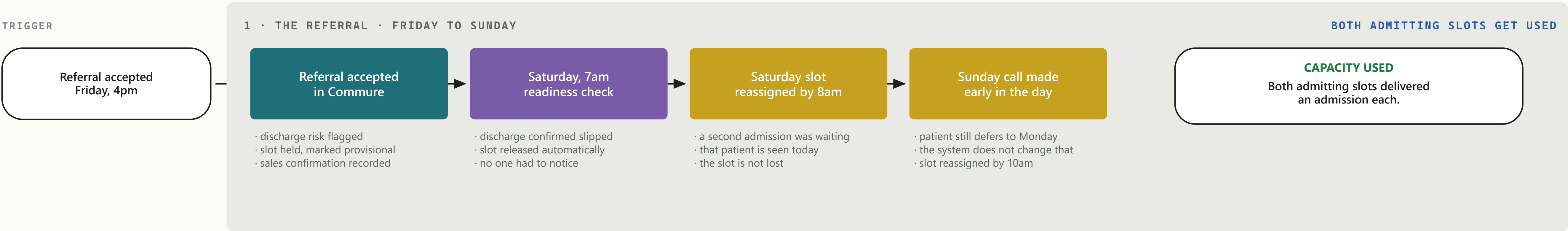
PCC / Scheduler

Clinician
- DCS

HCHB

Patient

TRIGGER



WHAT CHANGED, IN ORDER

1 ADMITTING TIME

- released early, reassigned
- to waiting admissions

2 THE PLAN OF CARE

- written with both in view
- at the moment of decision

3 THE CANCELLED VISIT

- refilled in minutes, income
- and capacity both kept

4 THE TWO DEADLINES

- both surfaced while days
- still remained

LEVERS ON THIS SHEET

- L1 Admission Throughput
- L3 Same-Day Schedule Recovery
- L4 Episode Payment Protection
- L5 Reassessment Window Compliance

One clinician's week

A registered nurse who can admit, paid per visit. Monday to Friday, and what the week actually paid her.

- Intake

Insurance & Auth

PCC / Scheduler

Clinician

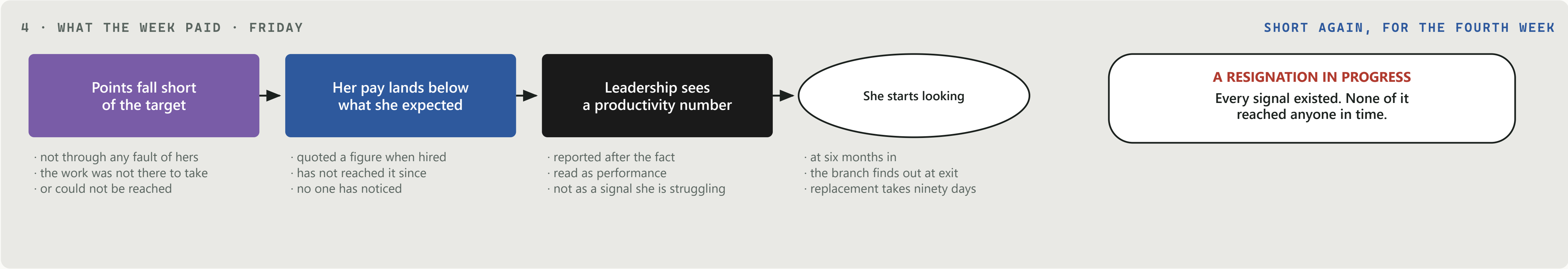
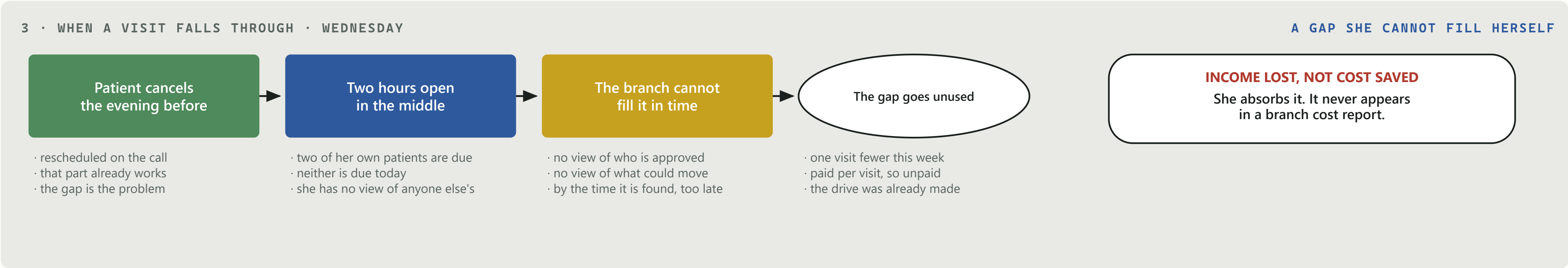
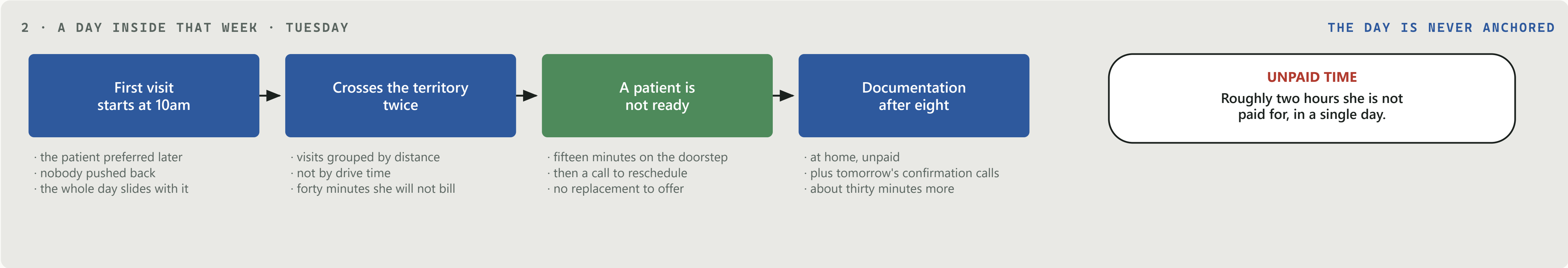
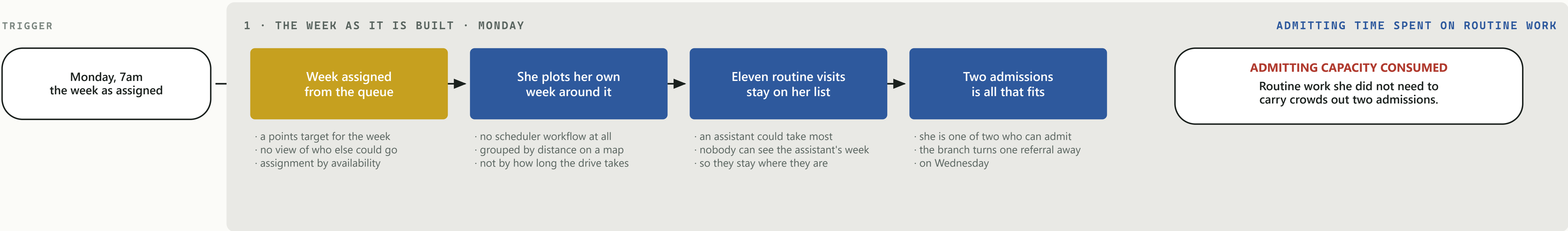
DCS
- HCHB

Per Diem / Float

Patient

Branch leadership

TRIGGER



WHAT THE WEEK COST HER, AND US

1 ADMITTING TIME

- routine work she did not need to carry

2 THE SHAPE OF THE DAY

- a ten o'clock start and two territory crossings

3 THE CANCELLED VISIT

- a gap she could not fill, absorbed as lost income

4 WHAT IT PAID

- short again, and nobody saw it happening

LEVERS ON THIS SHEET

- L2

Assessment Capacity Release
- L3

Same-Day Schedule Recovery
- L8

Clinician Retention
- L9

Travel and Territory Efficiency

One clinician's week — the same week, after

The same nurse, the same caseload, and what changes around her.

- Intake

HCHB
- Insurance & Auth

Per Diem / Float
- PCC / Scheduler

Patient
- Clinician

Branch leadership
- DCS

TRIGGER

Monday, 7am
the week as assigned

1 · THE WEEK AS IT IS BUILT · MONDAY

Week proposed
with her caseload

- routine work offered to assistants
- only where they have room
- she reviews and adjusts

She still plots
her own week

- the tool recommends, she accepts
- nothing is assigned to her
- grouped by real drive time

Seven routine visits
move to an assistant

- who had the capacity
- and the right qualification
- visible for the first time

Four admissions
now fit

- two more than before
- the Wednesday referral is accepted
- no one was hired

ADMITTING TIME PROTECTED

ADMITTING CAPACITY RELEASED
Two more admissions from the
same nurse and the same week.

2 · A DAY INSIDE THAT WEEK · TUESDAY

Confirmation handled
the evening before

- not by her, and not unpaid
- a nine o'clock start negotiated
- the patient agreed to it

First visit at nine
anchors the day

- every visit after it holds
- no slide, no late finish
- the single largest lever she has

Route follows
drive time

- not distance on a map
- one crossing instead of two
- forty minutes back in the day

Documentation
inside the day

- the evening is her own
- no confirmation calls to make
- roughly two hours returned

THE DAY IS ANCHORED AT NINE

TIME RETURNED
About two hours a day, and the
evening back.

3 · WHEN A VISIT FALLS THROUGH · WEDNESDAY

Patient cancels
the evening before

- rescheduled on the call
- unchanged from today
- the gap is now visible

Options ranked
immediately

- approval and discipline checked
- drive time from where she is
- flexible visits identified

Three offers
on her phone

- she chooses, or declines all
- accepted in eight minutes
- including one moved forward

The gap is filled

- the week stays whole
- the patient is seen sooner
- the later gap has days to fill

THE GAP IS OFFERED BACK IN MINUTES

INCOME PROTECTED
The week holds, and so does
her paycheck.

4 · WHAT THE WEEK PAID · FRIDAY

Points land on
target

- the work was there
- and it was reachable
- for the fourth week running

Pay matches what
she was quoted

- the number recruiting gave her
- is the number she earns
- at ninety days and at a year

Leadership sees
the trend, not the total

- a dip is visible in week one
- read as a signal, not a score
- a conversation, not a review

She stays

- the reason she leaves is removed
- not persuaded, just paid
- the caseload holds

THE WEEK LANDS WHERE IT SHOULD

A RESIGNATION AVOIDED
The signals reached someone
while it still mattered.

WHAT CHANGED

1 ADMITTING TIME

- moved to assistants with room,
- two more admissions fit

2 THE SHAPE OF THE DAY

- anchored at nine, routed by
- drive time, evening returned

3 THE CANCELLED VISIT

- offered back in minutes,
- she still chooses

4 WHAT IT PAID

- on target, and the trend is
- visible while it matters

LEVERS ON THIS SHEET

- L2** Assessment Capacity Release
- L3** Same-Day Schedule Recovery
- L8** Clinician Retention
- L9** Travel and Territory Efficiency

One branch's week

A branch running at capacity. The queue, the call-out, the referral it had to turn down, and what the week added up to.

- Intake

HCHB
- Insurance & Auth

Per Diem / Float
- PCC / Scheduler

Patient
- Clinician

Branch leadership
- DCS

Trigger

Monday, 7am
the queue as it stands

1 · The Queue · Monday Morning

The queue rebuilds overnight

- a task for every discipline and again at approval
- plus one per patient awaiting approval

The scheduler opens them one at a time

- most need no action at all but each must be opened to know the day is spent finding out

Bulk clearing becomes the habit

- a rational response to the volume the one that mattered goes with it nobody can tell which

Real decisions get the time that is left

- the welcome call is one of them it slips to the afternoon or to tomorrow

Most of the day is not a decision

ADMINISTRATION, NOT SCHEDULING
The judgment work is squeezed into what the queue leaves over.

2 · The Call-out · Wednesday, 7am

A clinician calls out at seven

- six visits on her day two are time-sensitive four could move if someone knew

Nobody can see who has room

- capacity lives in a spreadsheet updated when someone remembers and not this morning

The branch calls around

- per diem first, then contract then incentive pay for extra work whoever answers first

Covered, at a premium

- two visits covered this way two moved, two simply missed the cost lands in the month

The most expensive option is the fastest

PREMIUM SPEND, BOUGHT IN A HURRY
Not because coverage was scarce. Because it could not be seen.

3 · A Referral Arrives · Thursday

Referral offered by the hospital

- a good fit for the branch the kind we want an answer needed today

Both admitting nurses look full

- their weeks are full of routine visits an assistant could take many no view of the assistant's week

The branch declines

- a defensible call on what is visible the wrong call on what is true the source notices

Capacity existed

- it was in the wrong hands and nobody could see it the referral goes elsewhere

Turned down with capacity in the building

AN ADMISSION LOST
Declined for want of capacity the branch actually had.

4 · The Week in Review · Friday

The branch reviews the week

- visits delivered, points, census all of it accurate none of it showing what was lost

Premium spend posts to the month

- without the reason attached read as a staffing problem not a visibility problem

The declined referral is not recorded

- no reason code exists for it so it never happened and cannot be counted

The week looks fine

- every number is defensible the losses are all invisible so nothing changes next week

None of this appears as a number

INVISIBLE, THEREFORE PERMANENT
What cannot be counted cannot be managed, or improved.

What the week cost the branch

1 The Queue

- a day spent opening items that needed no action

2 The Call-out

- premium bought four times because nothing was visible

3 The Referral

- declined for want of capacity the branch actually had

4 The Review

- every loss invisible, so nothing changes next week

Levers on this sheet

- L1 Admission Throughput
- L2 Assessment Capacity Release
- L6 Scheduling Administration Cost
- L7 Premium Labor Avoidance

One branch's week — the same week, after

The same branch, the same week, and what it is able to do instead.

- Intake

HCHB
- Insurance & Auth

Per Diem / Float
- PCC / Scheduler

Patient
- Clinician

Branch leadership
- DCS

TRIGGER

Monday, 7am
the queue as it stands

1 · THE QUEUE · MONDAY MORNING

Notifications fire on change only

- not on the same state each day
- the volume falls away
- what remains needs an answer

The scheduler works exceptions

- urgency, coverage, local knowledge
- the work only a person can do
- fewer people, better used

Welcome calls go out first thing

- standardised, and early
- before the day fills
- readiness known while it is useful

Care team assigned once, at referral

- not once per discipline
- the task burst never forms
- approval is a single step

THE QUEUE ONLY CARRIES DECISIONS

SCHEDULING, NOT ADMINISTRATION
The queue holds decisions, and the day holds judgment work.

2 · THE CALL-OUT · WEDNESDAY, 7AM

A clinician calls out at seven

- the same six visits
- the same two time-sensitive
- nothing about this changes

The branch can see who has room

- live, across the whole branch
- approval and discipline checked
- including flexible visits

Two covered from existing capacity

- clinicians already on the payroll
- who had room and were near
- offered, and they accepted

Premium used once, not four times

- for the visit that genuinely
- had no other option
- the rest cost nothing extra

COVERAGE IS FOUND BEFORE IT IS BOUGHT

PREMIUM SPEND, PLANNED
Bought where it was needed, not where it was quickest.

3 · A REFERRAL ARRIVES · THURSDAY

Referral offered by the hospital

- the same referral
- the same day
- the same people available

Admitting capacity shown honestly

- routine work that could move
- assistants with room to take it
- the true number, not the visible one

Routine visits shift to assistants

- only where they have capacity
- and the right qualification
- the nurse reviews and accepts

The referral is accepted

- an admission the branch would
- otherwise have declined
- no one was hired

ACCEPTED, USING THE SAME PEOPLE

AN ADMISSION GAINED
The capacity was always there. Now it can be seen and used.

4 · THE WEEK IN REVIEW · FRIDAY

The branch reviews the week

- the same accurate figures
- plus the ones that were missing
- capacity offered against filled

Premium spend shows its reason

- reactive or planned, per event
- a visibility problem is visible
- and therefore fixable

Declines are recorded with a reason

- capacity, clinical, or geography
- counted, and trended
- the argument for hiring gets evidence

The week can be improved

- because it can be measured
- the same week, next week,
- is a decision, not a hope

THE LOSSES BECOME NUMBERS

MEASURED, THEREFORE MANAGEABLE
The first week the branch could see what it lost.

WHAT CHANGED

1 THE QUEUE

- a queue that only carries
- things needing a decision

2 THE CALL-OUT

- premium bought once, where
- it was genuinely needed

3 THE REFERRAL

- accepted, by moving routine
- work to assistants with room

4 THE REVIEW

- losses counted, so the week
- can actually be improved

LEVERS ON THIS SHEET

- L1** Admission Throughput
- L2** Assessment Capacity Release
- L6** Scheduling Administration Cost
- L7** Premium Labor Avoidance

The weekend cascade

Three weekend starts committed on Friday, and what each one did to the week that followed.

- Intake

Insurance & Auth

PCC / Scheduler

Clinician

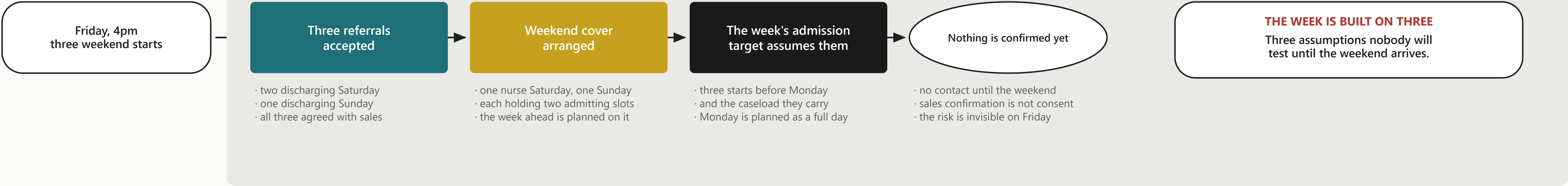
DCS
- HCHB

Per Diem / Float

Patient

Branch leadership

TRIGGER



The weekend cascade — the same week, after

The same three referrals, the same patient decisions, and a different Monday.

- Intake

HCHB
- Insurance & Auth

Per Diem / Float
- PCC / Scheduler

Patient
- Clinician

Branch leadership
- DCS

TRIGGER

Friday, 4pm
three weekend starts

1 · FRIDAY · THREE WEEKEND STARTS COMMITTED

Three referrals accepted

- the same three referrals
- the same expected discharges
- nothing about Friday changes

Each start carries a readiness state

- confirmed, expected, or at risk
- based on discharge patterns
- and on what sales recorded

Weekend cover arranged the same

- one nurse Saturday, one Sunday
- but the slots are now provisional
- and a waiting list sits behind them

A second call is queued behind each

- patients who could start early
- if a slot came free
- nobody had to build that list by hand

THE SAME THREE, WITH THE RISK NAMED

THE WEEK IS BUILT ON THREE,
with something ready to take
the place of any that fall.

2 · SATURDAY

Readiness checked at seven

- automated, before the day starts
- the late discharge is already known
- no one had to chase it

First patient discharged late

- exactly as before
- the patient's situation is unchanged
- only our knowledge of it changed

The slot is offered on by eight

- to the waiting patient behind it
- who accepts for that morning
- the nurse's day stays full

Two starts delivered

- one planned, one recovered
- from a slot that would have expired
- the same nurse, the same shift

TWO STARTS, FROM THE SAME TWO SLOTS

A FULL SATURDAY
The late discharge cost nothing
except a different patient's order.

3 · SUNDAY

Welcome call made at eight

- standardised, and early
- not waiting on office capacity
- four hours earlier than before

The family still defers to Monday

- the same reasonable decision
- no system changes that
- but we know at eight, not at noon

Both slots reassigned by ten

- to patients ready today
- with the whole day still ahead
- the nurse works a full Sunday

Two starts on Sunday

- not the ones planned on Friday
- the slots were used, which is
- what the week actually needed

THE DEFERRAL STILL HAPPENS. THE DAY DOES NOT.

A SUNDAY THAT HELD
The deferral was never the problem.
Finding out too late was.

4 · MONDAY, AND THE WEEK THAT FOLLOWS

One admission lands on Monday

- the deferred patient, as agreed
- four already started
- Monday was planned for two

Routine visits run as planned

- nothing pushed into Tuesday
- nothing compressed
- no window put at risk

Thursday's referral is accepted

- the branch has genuine room
- because the weekend did its work
- the source is told yes

The target is met

- from the same demand
- and the same people
- with four weekend starts, not one

NO CASCADE

ONE WEEKEND, ONE WHOLE WEEK
The weekend set the week up
instead of setting it back.

WHAT THE WEEKEND GAVE THE WEEK

1 FRIDAY

- the same three, each carrying
- a readiness state and a backup

2 SATURDAY

- the slot is offered on by eight,
- the day stays full

3 SUNDAY

- an eight o'clock call, the same
- deferral, both slots reused

4 THE WEEK

- one lands Monday, the week runs
- as planned, the referral accepted

LEVERS ON THIS SHEET

- L1 Admission Throughput, in depth

Scheduler's Recovery — the Nurse Calls Out

Wednesday morning. The SN from yesterday's plan calls out sick. Her five visits — SOC, OASIS discharge, three routine — are now stranded, and the scheduler has to recover the day before the first door.

STRANDED CASELOAD — 5 VISITS · 7.25 PTS

SOC · Mrs. Alvarez

2.5 · RN · West Ashley

OASIS D/C · Mr. Bell

1.75 · RN · Mt Pleasant

Routine · Mrs. Chen

1.0 · wound · West Ashley

Routine · Mrs. Evans

1.0 · CHF/dementia · Johns Is.

Routine · Mr. Davis

1.0 · MRSA · Downtown

01

Log the call-out

Open the coverage workflow; confirm scope — out today, and is tomorrow at risk too?

→

02

Triage by hard deadline

Which visits can't slip (SOC window, discharge order) vs. reschedulable inside their frequency window.

→

03

Read today's open capacity

Other RNs, per-diem / float; assessing-RN capacity specifically for the OASIS visits.

→

04

Cover the non-negotiable, slide the rest

Match discipline · competency · geography · continuity for anything that must happen today.

→

05

Notify & re-confirm

Patients, caregiver-present windows, and the care team — before anyone drives.

→

06

Rebalance & document

Protect covering clinicians' ceilings; check Thursday's load; missed-visit notes only if truly uncovered.

1

COVER TODAY

2.5 pts

SOC · Mrs. Alvarez

29407 · West Ashley · 2.5 pts

Assign per-diem RN (Nia P.) — AM opening, West Ashley territory. Re-confirm the 8–11a caregiver window with the new nurse and call the daughter now.

REGULATION

SOC 48-hr window expires TODAY — cannot slip; OASIS comprehensive + F2F required

COVERAGE

Assessing-RN only — pull from per-diem / float; confirm assessing capacity today

PATIENT / CAREGIVER

Caregiver home 8–11a → new nurse must hit that window; call the daughter

GEOGRAPHY

West Ashley — match a near-territory RN to protect the morning slot

COST

Per-diem = premium labor; justified by the hard SOC deadline

CONTINGENCY — IF NO RN CAN TAKE THE SOC IN THE CAREGIVER WINDOW

Escalate to the DCS: **(1)** the DCS (an RN) performs the SOC herself · **(2)** borrow an RN from a sister branch · **(3)** last resort — the SOC slips: document, notify the referral source, and flag the at-risk admission. This is the one visit that must not be dropped.

2

RESCHEDULE THU

1.75 pts

OASIS Discharge · Mr. Bell

29464 · Mt Pleasant · 1.75 pts

Slide to Thursday AM — within the OASIS-Discharge window (≤ 2 days of the discharge date). Hold for the nurse's return; notify Mr. Bell.

REGULATION

OASIS D/C allowed within 2 days of discharge → Thursday is in-window

COVERAGE

RN required — keep for the nurse to avoid a substitute doing the discharge OASIS

GEOGRAPHY

Mt Pleasant (far east) — batch with another east-side visit Thursday

3

RESCHEDULE THU

1.0 pt

Routine · Mrs. Chen — wound

29414 · West Ashley · 1.0 pt

Reschedule to Thursday AM (her preference) — 3×/wk spacing still met, no frequency gap. Confirm dressing supplies on hand.

REGULATION

Ordered 3×/wk — spacing holds if seen Thursday; no gap exception

COVERAGE

Would need wound competency to cover today — not worth a substitute for one day

PATIENT / CAREGIVER

Prefers mornings — offer Thu AM

GEOGRAPHY

West Ashley

4

RESCHEDULE THU

1.0 pt

Routine · Mrs. Evans — CHF (dementia)

29455 · Johns Island · 1.0 pt

Reschedule to Thursday with the same nurse — continuity matters for dementia. Confirm the husband is present; call ahead to reassure (anxious).

CONTINUITY

Dementia → a substitute is disruptive; keep the same nurse → reschedule, don't cover

REGULATION

Ordered 1×/wk — inside the window through Saturday

PATIENT / CAREGIVER

Husband must be present; anxious patient — proactive call

COVERAGE

If a CHF check is acuity-flagged, send a covering RN today instead

5

RESCHEDULE THU

1.0 pt

Routine · Mr. Davis — med mgmt (MRSA)

29403 · Downtown · 1.0 pt

Reschedule to Thursday (a non-dialysis day). Sequence last again; PPE.

PATIENT / CAREGIVER

Dialysis Mon/Wed/Fri 1–4p → he's unavailable this afternoon anyway

REGULATION

Ordered within window; contact isolation → sequence last

GEOGRAPHY

Downtown

COST

Lowest urgency — safe to slide

THEN THE SCHEDULER ALSO...

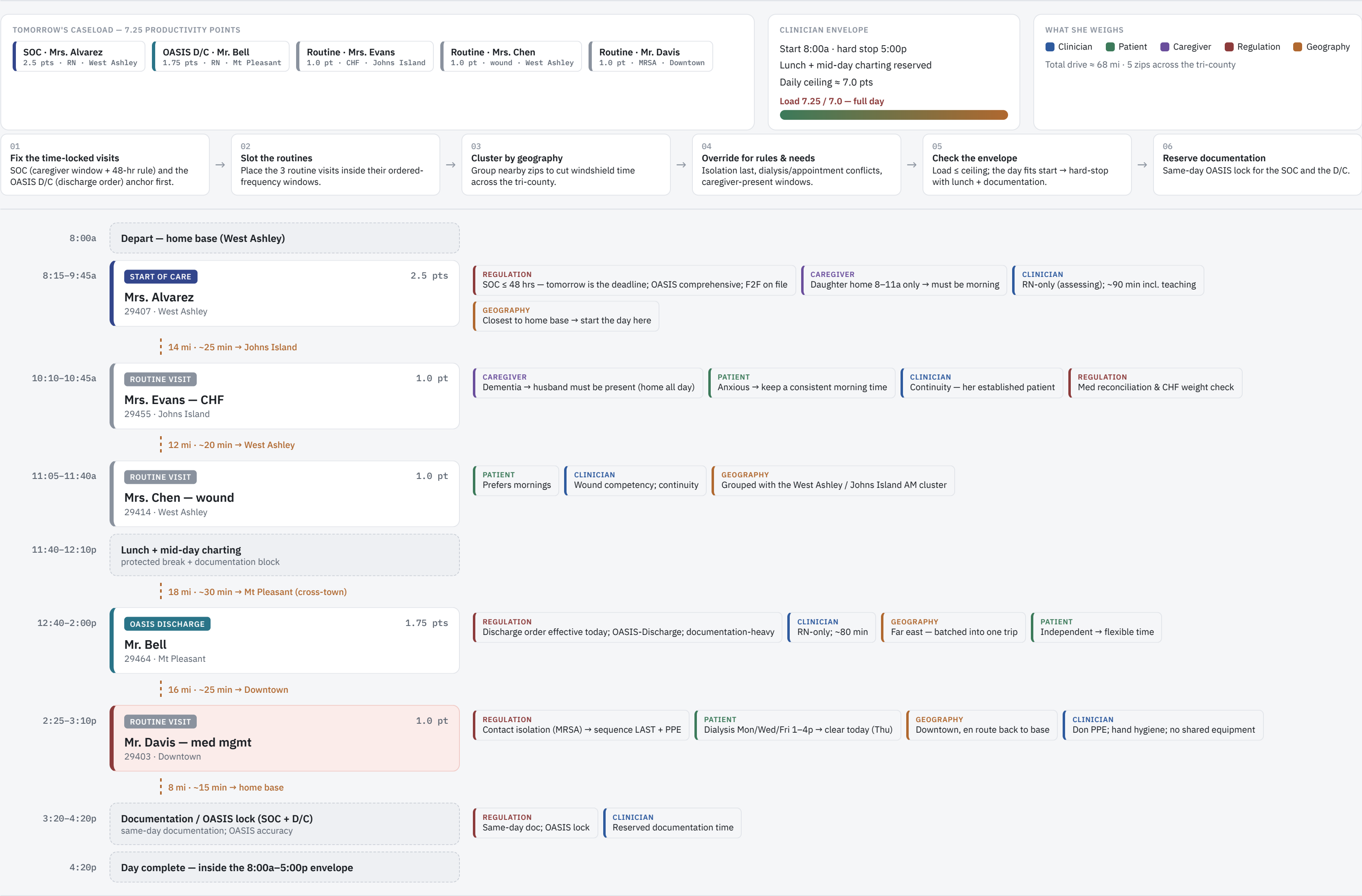
Confirms whether the nurse is out **tomorrow too** — if so, Thursday's four slid visits need real coverage, not just her return. Re-checks **Thursday's load** (her existing caseload + 4 slid visits can blow the ~7-pt ceiling → distribute or offload to per-diem). Updates the roster / availability and logs the sick day. **No missed-visit documentation** — everything is covered or rescheduled in-window; notes are only filed if a visit truly goes undelivered.

Outcome: SOC covered today (per-diem RN) · D/C + 3 routines rescheduled to Thursday in-window · 5 patients & caregivers notified · Thursday load rebalanced · zero missed-visit exceptions.

Illustrative recovery built from the Capacity & Scheduling variable model (coordination & coverage layer). Fictional patients; the disposition logic is the real path a scheduler runs on a same-day call-out.

A Nurse's Next Day — Building the Schedule

One SN, five visits: 1 Start of Care · 1 OASIS Discharge · 3 routine. Every visit tagged with what she has to weigh — clinician, patient, caregiver, regulation, geography.



Branch Leader Review — Evaluating Nurse Territories

A quarterly read on demand, capacity, and coverage. The ED overlays three months of trends per nurse and territory, then decides where to redraw zips, staff up, or rebalance. Example: Charleston Tri-County, 4 SN territories.

THE 3-MONTH PICTURE — THE SIGNALS SHE PULLS FIRST

Referral distribution

Shifting West / South

New-referral volume migrating to West Ashley + Johns Island growth corridors; peninsula flat-to-down.

Per-diem utilization

▲ 40%

• 140 hrs/mo

Premium coverage climbing 3 months straight — a structural signal, not a one-off spike.

Caseload spread

1 over · 1 under

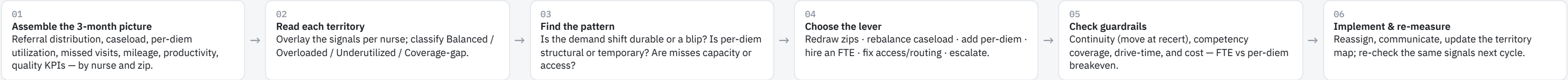
Imbalanced: Torres 112% (over ceiling), Blake 78% (slack). Room to rebalance internally.

Missed-visit rate

5.4% branch · 9.2% peak

Rising, concentrated in Downtown/peninsula — access & geography, not raw capacity.

THE EVALUATION FLOW



TERRITORY DIAGNOSIS

West Ashley — RN Torres

29407 · 29414

OVERLOADED

CASELOAD112%

REFERRALS 3-MO▲ 18%

PER-DIEM62 hrs ▲

MISSED6.8% ▲

OVERTIME▲

RECOMMENDED LEVER

Shed a zip + queue an FTE. Move 29414 to Blake to pull her under ceiling now. The rising per-diem here is paying premium for **durable** demand growth — earmark this territory for the new hire.

Mt Pleasant / East — RN Blake

29464 · 29466

UNDERUTILIZED

CASELOAD78%

REFERRALS 3-MOflat

PER-DIEM8 hrs

MISSED2.1%

MILEAGElow

RECOMMENDED LEVER

Absorb 29414 from Torres. Clear slack and clean reliability — the best home for the rebalance, and positioned to take rising east-side referrals without new cost.

Downtown / Peninsula — RN Diaz

29401 · 29403

COVERAGE / RELIABILITY GAP

CASELOAD96%

REFERRALS 3-MO▼ slight

PER-DIEM40 hrs ▲

MISSED9.2% ▲

MILEAGEhigh

RECOMMENDED LEVER

Fix access, not headcount. Misses are no-access + parking, not overload — add day-before confirmation discipline and tighten the route; a downtown-only per-diem for surge. No redraw yet — watch one more cycle.

Johns Island / Rural — RN Ellis

29455 · 29487

GROWING · DRIVE-TIME RISK

CASELOAD88%

REFERRALS 3-MO▲ 14%

PER-DIEM30 hrs ▲

MISSED4.5%

MILEAGEvery high

RECOMMENDED LEVER

Base the new FTE south. Rural demand rising with long drives; a satellite per-diem or the new hire based on Johns Island caps mileage and per-diem. Guard continuity as the territory grows.

BRANCH DECISION & GUARDRAILS

The signals converge: demand is migrating west and south, per-diem is up 40% and structural, one nurse is over and one is under, and peninsula misses are an access problem — not a staffing one. So:

- Redraw — move zip 29414 from Torres to Blake to level the over/under today.
- Hire 1 FTE RN, based south — three months of climbing per-diem in the West Ashley + Johns Island growth corridors is cheaper to internalize than to keep renting; model the FTE-vs-per-diem breakeven before posting.
- Peninsula = process fix — confirmation calls + route tightening for Diaz, plus a downtown per-diem for surge. No headcount.
- Guardrails — phase patient moves at recert to protect continuity; keep the wound-competent RN on West Ashley; hold drive-time under target; don't push Blake past ceiling.

Re-measure next cycle: per-diem hours, missed-visit rate by zip, caseload balance, and mileage — did the redraw + hire bend the curves?